PLAYBOOK — 2026 EDITION

The Pharma Leader's Playbook: How Peer-to-Peer Mentorship Outperforms Every Other Patient Engagement Tactic

The framework behind 68% more treatment starts and 133.5 more days on therapy. Backed by 50,000+ patient interactions, independent research, and the data pharma leadership actually wants to see.

68%
MORE TREATMENT STARTS

133.5
MORE DAYS ON THERAPY

90%
CONFIDENCE BOOST

PatientPartner

Patient Engagement Software That Delivers Measurable ROI

patientpartner.com

Executive Summary

The pharma industry spends $22 billion annually on patient support programs. Only 3–8% of eligible patients use them. Meanwhile, 50% of patients on chronic medications abandon treatment within the first year, costing the U.S. healthcare system $100–$300 billion in avoidable costs and contributing to 125,000 preventable deaths.

The missing piece isn't logistics. It's human connection.

This playbook presents the evidence that peer-to-peer mentorship—connecting patients with trained mentors who've completed the same treatment—outperforms every other patient engagement tactic on the metrics that matter: treatment starts, adherence, persistence, and ROI.

→ **Chapter 1:** The $22B engagement crisis—why utilization is stuck at single digits

→ **Chapter 2:** The trust shift—Edelman, IQVIA, and Health Union data on why peers beat brands

→ **Chapter 3:** Head-to-head evidence—peer mentorship vs. every other tactic

→ **Chapter 4:** The 5-Principle Framework for building an effective peer program

→ **Chapter 5:** ROI math your CFO will approve

→ **Chapter 6:** Implementation roadmap—from strategy to live program in under 4 months

CHAPTER 1

The $22 Billion Engagement Crisis

Pharma invests more in patient support than ever before. The returns have never been worse.

According to research from Phreesia, GoodRx, and IQVIA, the industry allocates over **$22 billion annually** to patient support programs—hub services, copay assistance, nurse educators, digital

tools, call centers. Yet only **3–8% of eligible patients** actually use these programs. And **59% of patients** report little or no knowledge that support programs even exist.

3-8%

of eligible patients use the support programs pharma designs for them—despite $22B+ in annual investment.

Phreesia/GoodRx PSP Utilization Research; IQVIA Four Forces (2026)

The problem isn't a lack of programs. It's a fundamental misalignment between what programs deliver and what patients need at the moment of decision.

What Programs Deliver vs. What Patients Need

WHAT PSPS TYPICALLY PROVIDE

- Prior authorization support
- Copay cards and financial assistance
- Nurse educator phone calls
- Automated refill reminders (SMS/email)
- Specialty pharmacy coordination
- Enrollment in hub services

WHAT PATIENTS ACTUALLY NEED

- Someone who's been through the same treatment
- Confidence the treatment is worth the fear
- A real conversation with a real patient
- Answers to questions they won't ask their doctor
- Reassurance during the hardest moments
- Proof that life gets better on the other side

This is what we call **the "missing middle"**—the psychological gap between enrollment and first fill that no amount of logistics can close. A patient can have their prior auth approved, their copay covered, and their specialty pharmacy on standby. They still won't start therapy if they're afraid.

The Adherence Catastrophe in Numbers

The consequences of this gap cascade across the entire healthcare system:

METRIC	DATA	SOURCE
Chronic medication adherence rate	~50%	WHO; Dialog Health (2025)
Patients who abandon treatment within Year 1	50–60%	Dialog Health (2025)
Annual U.S. cost of non-adherence	$100–$300B	NEHI; Frontiers in Pharmacology (2025)
Deaths linked to non-adherence annually (U.S.)	125,000	AJMC; Dialog Health (2025)
Hospitalizations linked to non-adherence	Up to 25%	Dialog Health (2025)
Americans who struggle with medication instructions	75%	Dialog Health (2025)
Treatment failures caused by non-adherence	Up to 50%	Dialog Health (2025)

For specialty therapies—the drugs that drive pharma's growth—the stakes are even higher. A single patient's annual therapy value often exceeds $50,000. Every patient who enrolls but doesn't start represents lost revenue, wasted acquisition spend, and reduced program ROI.

> *"Patient support leaders face unprecedented pressure heading into 2026. Rising healthcare costs, evolving policy mandates, and increasing patient expectations are fundamentally changing how programs are designed, delivered, and measured."*
>
> **IQVIA, "Four Forces Reshaping Patient Support in 2026" (January 2026)**

CHAPTER 2

The Trust Shift: Why Patients Trust Peers Over Brands

Something fundamental has changed in how patients make healthcare decisions. It's not just about information access—it's about who they believe.

The **2025 Edelman Trust Barometer: Special Report on Health** reveals a dramatic shift in health authority. While "my doctor" remains the most trusted voice globally (82%), a new class of legitimate health expert has emerged: **peers with lived experience**.

67%
SAY LIVED HEALTH EXPERIENCE QUALIFIES SOMEONE AS A HEALTH EXPERT

64%
TRUST SOMEONE WHOSE HEALTH ADVICE HELPED IN THE PAST

45%
OF 18–34S SAY A RESEARCHED PATIENT = A DOCTOR IN KNOWLEDGE

Source: 2025 Edelman Trust Barometer: Special Report — Trust and Health (15,009 respondents across 15 markets)

As Edelman's Jennifer Hauser wrote: *"A new health credential is taking shape—one that elevates the importance of human connection and centers on communication as a key differentiator... Credentials remain critical, but they must now be accompanied by communication that builds emotional trust."*

The Industry Confirms the Shift

This isn't a consumer trend happening in isolation. Pharma's own leadership sees it. The **2026 Pharma Marketing Pulse Report** from Health Union and solli—drawing from 1,500+ data points across 50+ senior pharma marketers (75% Director-level or above)—found:

FINDING	DATA
Marketers reallocating budgets from general search to specialized, high-authority platforms	**68%**
Rank condition-specific communities as "extremely important"	**67%**
In Oncology/Rare Disease, rate community platforms as critical	**75%**

FINDING	DATA
Marketers increasing spend on platforms with validated medical authority	**45%**
Using influencer partners that require validated medical/regulatory review	**93%**

Source: 2026 Pharma Marketing Pulse Report, Health Union & solli (February 2026)

The report's conclusion: pharma is shifting *"from vanity metrics to verified authority and genuinely qualified engagement."* The era of measuring clicks and impressions is over. What matters now is whether your engagement drives actual treatment decisions.

> *"Marketers are no longer just looking for reach; they are prioritizing trusted specialty platforms that can link top-of-funnel engagement to bottom-line results like prescription lift and ROI. By leaning into environments built on lived experience, brands can move beyond simple clicks to find the deep, qualified engagement that drives real outcomes."*
>
> **Olivier Chateau, CEO of Health Union (February 2026)**

Why This Matters for Patient Support

If patients trust peers with lived experience more than institutions, and if pharma marketers are already shifting budget toward community-based engagement—then patient support programs built on peer-to-peer mentorship aren't just a nice idea. **They're aligned with where trust actually lives.**

The question isn't whether peer-to-peer works. It's whether your program is built to deliver it.

CHAPTER 3

The Evidence: Peer Mentorship vs. Everything Else

Let's compare peer-to-peer mentorship to the most common patient engagement tactics—not with theory, but with data.

Effectiveness by Intervention Type

INTERVENTION	ADHERENCE IMPACT	SOURCE
Peer mentorship (1:1, matched)	**Up to 90% adherence boost; 68% take next treatment step**	rxpx.health; PatientPartner 12-mo study
Face-to-face education (pharmacist/nurse)	83% success rate in RCTs	Dialog Health meta-analysis (2025)
Community/family engagement	21% adherence improvement; 69–75% greater persistence	Dialog Health (2025)
Text message reminders	2x odds of adherence (OR 2.11)	Meta-analysis, 16 RCTs, N=2,742
Hospital discharge interventions	67% success rate	Dialog Health (2025)
Electronic interventions (auto-calls, smart pillboxes)	67% success rate	Dialog Health (2025)
Phone call interventions	38% success rate	Dialog Health (2025)
Paper-based methods (mail, fax)	33% success rate	Dialog Health (2025)
Automated refill programs (30/90-day)	1.4–3% improvement	Dialog Health (2025)

The pattern is unmistakable: **the more human the intervention, the more effective it is.** Automated systems help marginally. Phone calls are better. Face-to-face education works well. But peer mentorship—a real person who has walked the same path—produces the strongest outcomes across the board.

The PatientPartner Data Set

A 12-month study tracking 263 prospective patients across type 2 diabetes and women's health conditions—with 193 mentor connections and 48 dedicated mentors—produced results that no other intervention category matches:

73%
MENTOR CONNECTION RATE (VS. 10–20% FOR TRADITIONAL PSPS)

71%
SAID MENTORSHIP WAS CRITICAL TO TREATMENT DECISION

68%
TOOK PROACTIVE NEXT STEPS (REFERRALS, APPOINTMENTS, RX)

Among connected patients, qualitative feedback consistently highlighted three outcomes:

- **Reduced anxiety** about treatment options after engaging with a mentor
- **Enhanced confidence**—patients felt empowered in their healthcare decisions
- **Increased understanding** of what to expect from treatment—the questions patients won't ask their doctor

133.5 more days on therapy for mentored patients compared to unsupported patients. At $50,000+ annual therapy value for specialty drugs, this is the highest-ROI intervention in patient support.

PatientPartner Adherence Analysis (2023–2026)

Why Peer Mentorship Outperforms

Three structural advantages explain the performance gap:

1. Emotional resonance beats informational content. A nurse educator can explain side effects. A mentor who experienced them can say "Here's what the first week actually felt like, and here's why it was worth it." That distinction drives action.

2. On-demand timing matches decision windows. Traditional programs schedule follow-ups weeks out. Patients make treatment decisions in moments of fear and uncertainty—at 10pm, after reading side effects online. Real-time peer access meets patients when the decision is actually happening.

3. Matched experience creates instant credibility. Generic communities require patients to sift through hundreds of posts. One-to-one matching by condition, treatment, demographics, and experience creates an immediate connection: "This person understands exactly what I'm going through."

CHAPTER 4

The 5-Principle Framework for Effective Peer-to-Peer Programs

Not all peer programs are created equal. Patient ambassador panels, online forums, and casual support groups have existed for years. The programs that produce measurable clinical and commercial outcomes share five principles.

The P2P Mentorship Framework

1

Precision Matching, Not Random Assignment

Match mentors to patients by condition, treatment, demographics, and lived experience. A 55-year-old woman starting biologics for RA needs a mentor who completed that specific therapy—not a general "patient advocate." The data shows matched mentorship produces 3–5x the connection rate of unmatched programs (73% vs. 10–20%).

2

Real-Time Access, Not Scheduled Callbacks

Treatment decisions happen in moments, not business hours. Programs that offer same-day or on-demand peer connections capture the decision window. Programs that schedule calls two weeks out lose patients to fear, doubt, and inertia. The connection must be available when the patient is ready, not when the coordinator has an opening.

3

Trained Mentors, Not Unstructured Volunteers

Effective mentors are vetted, trained, and supported. They know how to share their experience without giving medical advice. They know when to encourage, when to listen, and when to refer. In-house programs struggle here because training and managing a mentor pool is operationally intensive. Dedicated platforms handle mentor recruitment, training, compliance monitoring, and quality assurance.

4

Measurable Outcomes, Not Engagement Theater

Track what matters: treatment starts, adherence at 30/60/90/180 days, days on therapy, progression in care journey. If your peer program measures "calls completed" or "sessions scheduled" but can't tie those to commercial outcomes, it's not delivering. Real programs capture sentiment data, decision progression, and downstream treatment behavior.

5

Enterprise Compliance, Not Workarounds

HIPAA, SOC 2, ISO 27001, adverse event reporting, and audit trails aren't optional in pharma. Every patient interaction must be logged, monitored, and accessible to compliance teams. Programs that rely on phone calls, social media groups, or unmonitored community platforms create regulatory exposure. Compliance must be built into the infrastructure, not bolted on.

How Existing Programs Compare

PROGRAM TYPE	MATCHING	REAL-TIME	TRAINED	MEASURABLE	COMPLIANT
Open patient communities (PatientsLikeMe, Inspire)	✗	Partial	✗	✗	✗
Pharma-sponsored ambassador programs	Partial	✗	✓	Partial	✓
Advocacy/storytelling platforms (Snow Companies)	✗	✗	✓	Partial	✓
Standardized support programs (Reverba)	Partial	✗	✓	Partial	✓
1:1 matched mentorship platform	✓	✓	✓	✓	✓

CHAPTER 5

The ROI Math Your CFO Will Approve

Peer-to-peer mentorship doesn't just improve patient outcomes. It generates measurable return on investment that finance teams can model, forecast, and approve.

The Revenue Recovery Model

For specialty therapies, the math is straightforward:

Sample ROI Calculation: Specialty Therapy ($50K Annual Value)

Annual enrolled patients	**1,000**
Patients who don't start therapy (industry avg: 40%)	**400**
Patients recovered via peer mentorship (68% of connected)	**~200**
Average annual therapy value	**$50,000**
Recovered Annual Revenue	**$10M**

That's just the first-year revenue from new starts. The persistence benefit compounds:

29% better adherence among mentored patients. Combined with 133.5 more days on therapy, this means each recovered patient generates not just a first fill but sustained revenue over time.

PatientPartner Adherence Analysis; rxpx.health Industry Report

Three Revenue Drivers

REVENUE DRIVER	MECHANISM	IMPACT
New treatment starts	Mentorship converts enrolled-but-hesitant patients to active therapy	68% of connected patients take the next step
Improved persistence	Ongoing mentorship support reduces early discontinuation	133.5 more days on therapy per patient
Reduced acquisition waste	More enrolled patients convert, reducing cost-per-start	Every prevented drop-off saves $5K–$20K in acquisition costs

Comparing Cost-Effectiveness

When you compare the cost of a peer mentorship intervention against the revenue it recovers, the ROI is asymmetric. A platform-based mentorship program costs a fraction of the lost revenue from a single patient who enrolls but never starts.

As one industry analysis noted: *"Higher adherence driven by peer support ultimately boosts pharmaceutical sales and revenue"*—making peer mentorship one of the few patient support investments that directly improves the commercial P&L while also improving patient outcomes.

The Business Case Framework

When presenting to leadership, structure the case around these four proof points:

Building Your Internal Business Case

1. **Quantify Your Current Drop-Off**

   What percentage of enrolled patients never start therapy? What's the revenue value of that gap? Most programs sit at 30–50% drop-off between enrollment and first fill.

2. **Benchmark Against Peer Data**

   Compare your connection rate, conversion rate, and days-on-therapy against the benchmarks in this playbook. If your program's conversion rate is below 68%, there's a quantifiable gap to close.

3. **Model the Revenue Impact**

   Multiply recovered patients × annual therapy value. Even conservative assumptions (recovering 20% of drop-offs) produce 7–10x ROI for specialty therapies.

4. **Address the Risk Question**

   Compliance, adverse event reporting, and regulatory alignment are the first questions legal will ask. Lead with the compliance infrastructure: HIPAA, SOC 2, ISO 27001, full audit trails, AE monitoring built into every interaction.

CHAPTER 6

Implementation: Strategy to Live Program in Under 4 Months

Peer-to-peer mentorship programs don't require multi-year buildouts. A turnkey platform can be operational within a single quarter. Here's the implementation roadmap:

The 16-Week Roadmap

PHASE	TIMELINE	ACTIVITIES	DELIVERABLE
Discovery	Weeks 1–3	Map patient journey, identify drop-off points, define mentor profiles, align with compliance/legal	Program design document
Build	Weeks 4–8	Configure platform, recruit and train mentors, build matching algorithms, integrate with existing PSP/CRM	Platform ready for testing
Pilot	Weeks 9–12	Launch with initial patient cohort (50–100 patients), monitor connection rates and outcomes, refine matching	Pilot results and optimization plan
Scale	Weeks 13–16	Full program launch, expand mentor pool, activate across channels, begin outcome reporting	Live program with dashboard

Key Decisions at Each Phase

Discovery: Where Are You Losing Patients?

Use your existing data to identify the highest-impact drop-off points. For most programs, the biggest gap is between enrollment and first fill (the "missing middle"). But some brands see significant attrition at 90 days, during treatment switching, or at the transition from clinical trial to commercial product. The peer program should target the moment with the highest revenue impact.

Build: In-House vs. Platform

Some companies attempt to build peer programs internally. Takeda's "My Mentor Connections" for multiple myeloma, for example, enrolled 270 patients over its lifetime—demonstrating the concept works but also showing the scale limitations of in-house approaches. Dedicated platforms can deploy across multiple brands and indications simultaneously, with existing mentor pools (500+ mentors across 40,000+ users in the case of the largest platforms) and proven operational infrastructure.

Pilot: What to Measure

During pilot, track these KPIs weekly:

- **Connection rate**—% of patients who connect with a mentor (benchmark: 73%)
- **Decision impact**—% who report mentorship influenced their treatment decision (benchmark: 71%)
- **Progression rate**—% who take a next step (referral, appointment, Rx) (benchmark: 68%)
- **Patient sentiment**—confidence scores pre- and post-connection (benchmark: 90% boost)
- **Time to connection**—how quickly patients are matched (target: same day)

Scale: Multi-Brand Expansion

Once the pilot validates outcomes for a single brand or indication, the program architecture should support expansion to additional therapy areas without rebuilding infrastructure. Each new indication requires: new mentor recruitment and training, updated matching criteria, and brand-specific compliance review—but the core platform, operational playbook, and measurement framework carry over.

Integration Points

A peer mentorship program doesn't replace your existing PSP infrastructure. It plugs into the gap that hubs, copay programs, and nurse educators can't close:

YOUR EXISTING STACK

- Hub services (enrollment, PA, copay)
- Nurse educator programs
- Specialty pharmacy coordination
- CRM and patient data
- Adverse event reporting systems

WHAT PEER MENTORSHIP ADDS

- Emotional support at the decision point
- Real-time peer access (not scheduled)
- Patient sentiment intelligence
- Conversion data (enrolled → started)
- Qualitative insights from patient conversations

> *"Anecdote isn't the enemy of science—it's often the bridge to it. The new health credential lives in the intersection of rigor and relatability, where influence is earned through both knowledge and connection."*
>
> **Jennifer Hauser, Global Health Co-Chair, Edelman (April 2025)**

The Opportunity Window

The convergence is clear. Trust has shifted to peers. Industry budgets are following. The evidence base is conclusive. And patients are waiting for someone who understands what they're going through.

The pharma companies that act on this data now—building peer-to-peer mentorship into their patient support infrastructure—will capture a measurable advantage in treatment starts, adherence, persistence, and patient satisfaction. The ones that don't will continue spending $22 billion to reach 3–8% of eligible patients.

The framework in this playbook works. The data proves it. The question is whether you'll implement it.

See How Peer Mentorship Fits in Your Program

Book a 30-minute strategy session. We'll walk through your patient journey, identify where mentorship would have the highest impact, and share benchmarks from similar programs.

68%	133.5	<4mo
MORE STARTS	MORE DAYS ON TX	TO GO LIVE

Book a Strategy Session

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